SAMPLE ANSWER · STROKE · CASE 2

# Sample 7-minute ISBAR: Daniel

Daniel Okafor · 46-year-old man · Inpatient rehabilitation unit, public hospital · about 1018 spoken words (≈ 7.3 min at 140 words per minute)

## I Introduction

*54 words*

"Hi, I'm the occupational therapy student running Daniel Okafor's kitchen session. Daniel is a 46-year-old man, three weeks after a left middle cerebral artery ischaemic stroke. This is urgent. In the last few minutes his speech, face and right hand have suddenly worsened. I'm staying with him and I've pressed the emergency call bell."

## S Situation

*124 words*

"At about 10:35 am, while cutting a tomato, Daniel's speech became more slurred than at the start of the session. The right side of his face is drooping more, he dropped the knife and couldn't pick it up, and his right grip is much weaker than at 10:15. He says his head feels 'funny' and can't find words to explain.

Until 10:35 he was at his usual level, so that is the time he was last seen well. It's now 10:40. He says he's just tired, wants to keep going and has reached for a glass of water. The physio is at the door to take him to the gym. I need your help to get the medical emergency or stroke response here now."

## B Background

*144 words*

"Daniel had his stroke three weeks ago. His usual deficits are right arm weakness and mild word-finding difficulty, with good understanding. He'd been progressing well: he walks independently on the unit and his word-finding has been improving. He works full-time as an IT project manager and lives with his wife, Grace, and their two teenagers.

This morning a nurse mentioned his blood pressure had been 'up and down' over the last few days. At 10:15 Daniel told me he was tired after a bad night.

There are important gaps. I couldn't find his most recent blood pressure or blood glucose readings in the file, or what 'up and down' meant. I don't know whether he had any similar symptoms overnight, whether anything was reported to nursing staff, or which medications he's on. I'd want the responding team to clarify those rather than me assuming."

## A Assessment

*312 words*

"My assessment is that this is a sudden new change in Daniel's neurological status. The cluster of more slurred speech, increased facial droop and a sudden loss of right-hand grip, starting within a few minutes, matches the face, arm and speech warning signs of stroke. With his new head symptom and the reported blood pressure fluctuation, I'm concerned this could be a new or extending stroke. I can't diagnose that, and there are other possible causes, such as low blood sugar, but every one of them needs urgent medical assessment, and stroke is time-critical. Treatment options depend on how quickly he's assessed, which is why the time he was last seen well matters so much.

The immediate risks are these. First, delay: if we accept that he's 'just tired', we lose time. Second, aspiration: if his face and speech have worsened, his swallow may have too, so the water he's reaching for and the sandwich aren't safe until his swallow is checked. Third, falls: he's trying to stand to go to the gym, and his weakness may be worse than it was this morning.

I also want to name the things that could pull us off track. Daniel saying he's tired after a bad night is understandable, but tiredness doesn't explain a sudden one-sided change during a session I was watching. He's embarrassed and wants to keep going, and I want to respect that, but continuing the task isn't safe. The work emails, Grace's income protection forms and the physio timetable are all real, but none of them matters right now.

So my clear priority is getting medical assessment to Daniel immediately. His occupational goals, like returning to work, are important later, but they aren't the focus now. As a student, I'm not deciding on investigations or treatment; my job is to recognise the change, keep him safe and hand over accurately."

## R Recommendation

*384 words*

"My immediate action is to stay with Daniel, keep him seated safely, and explain calmly what's happening in short, simple sentences. I'd gently ask him not to eat or drink anything for now and move the water and food away, with the reason: we need to make sure swallowing is safe. I'd ask the physio to stay and help keep him safe, or to fetch the nurse, rather than taking him to the gym.

Second, I'd like your help to make sure the facility's medical emergency or stroke response is activated now, following our local procedure. When the team arrives, I'll hand over exactly: last seen well at 10:35, the specific changes in speech, face and grip compared with 10:15, his 'funny' head, and the reported blood pressure changes.

For the wider team, I'd ask the medical team to urgently assess him and decide whether he needs imaging or transfer to an acute stroke service. I'd ask nursing staff to do his observations, including blood pressure, blood glucose and neurological observations, and to keep him nil by mouth until his swallow has been reviewed. I'd ask speech pathology to reassess his swallow and communication once he's medically stable. Physiotherapy should postpone today's session. I'd also ask social work to support Grace once she's been contacted by the medical team, because this will be frightening for the family, and to pick up the income protection paperwork later.

From an OT perspective, once Daniel is medically stable, the principal short-term assessment I'd recommend is a repeat observed performance of the same toasted sandwich task. That gives a direct comparison with today's baseline and shows how any new change affects his function. For the longer term, I'd use the Canadian Occupational Performance Measure with Daniel and Grace, because his goals, like returning to work as a project manager, supporting his teenagers and family finances, will need re-prioritising after this setback.

The Stroke Foundation Living Clinical Guidelines support rapid assessment of anyone with suspected stroke, and F.A.S.T. is a recognised way to describe these signs.

Once the emergency is managed, I'd document my observations, the times and who I informed. I'd also like to debrief with you afterwards, because Daniel asked to keep going, and I want to talk about how I balanced his wishes with his safety."

## Coaching notes *not spoken*

### The immediate priority

A sudden worsening of speech, facial droop and grip during the session is a time-critical medical emergency. Stay with Daniel, note the time he was last seen well, keep him nil by mouth and get the facility's emergency or stroke response to him now.

### Distractors in this case

- **"I'm just tired after a bad night"** — fatigue does not explain a sudden one-sided change you have just watched happen.
- **Embarrassment and wanting to finish the task** — respect his feelings, but don't continue the session.
- **Work emails and income protection forms** — real stressors for later, not now.
- **Physio early for the gym session** — standing and exercise are unsafe until he has had a medical review; redirect the physio to help.

### Missing information to clarify

- Most recent blood pressure and blood glucose readings
- Whether he had similar symptoms overnight ("bad night")
- Current medications, including any antithrombotic therapy
- Whether nursing staff noted any change earlier this morning

### Useful references

- Stroke Foundation, *Living Clinical Guidelines for Stroke Management* (early assessment and acute care)
- Stroke Foundation, F.A.S.T. (Face, Arms, Speech, Time) stroke signs
- Local facility medical emergency/clinical deterioration procedure (NSQHS Recognising and Responding to Acute Deterioration Standard)

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